



Ascension

The Honorable Thomas Keane, MD, MBA
Assistant Secretary
Technology Policy/National Coordinator for Health Information Technology
200 Independence Ave SW
Washington, DC 20201

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Submitted electronically via: www.regulations.gov

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

Dear Assistant Secretary Keane:

Ascension appreciates the opportunity to submit our response to the Request for Information recently issued by the Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP) and Office of the National Coordinator for Health Information Technology (ONC) entitled *Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care*.¹

Global Comments

Ascension is one of the nation's leading non-profit and Catholic health systems, with a Mission of delivering compassionate, personalized care to all, with special attention to those most vulnerable. In FY2025, Ascension provided \$1.7 billion in care of persons living in poverty and other community benefit programs along with \$1.8 billion of unreimbursed care for Medicare patients. Across 15 states and the District of Columbia, Ascension's network encompasses approximately 97,000 associates, 23,100 aligned providers, and 91 wholly owned or consolidated hospitals. Ascension also operates 26 senior living facilities and a variety of other care sites offering a range of healthcare services.

Building upon this commitment to service and excellence, Ascension recently launched its Clinical Innovation Institute, a multidisciplinary initiative dedicated to reimagining patient care and the clinician experience through courageous innovation.² By uniting clinical expertise with digital transformation, our Institute focuses on vetting and implementing secure, cutting-edge technologies that reduce administrative burden and allow caregivers to prioritize direct patient engagement. Our mission-driven approach ensures that as we adopt artificial intelligence (AI) and other transformative tools, we remain steadfast in our commitment to safety, quality, and the delivery of holistic, patient-centered care.

¹ 90 Fed. Reg. 102,220 (Dec. 23, 2025)

² *Ascension Launches Clinical Innovation Institute Focused on Technology and Innovation in Patient Care and Clinician Experience*, Ascension (Aug. 4, 2025), <https://about.ascension.org/news/2025/08/ascension-launches-clinical-innovation-institute-focused-on-technology-and-innovation-in-patient-care-and-clinician-experience>.

Ascension's insights are below. We look forward to a continued dialogue on these critical advancements.

Questions and Responses

HHS asks the following:

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

Broadly, Ascension asserts that the current lack of a standardized, consensus-driven framework for patient education and consent presents a significant legal and operational throttle on consent and privacy. Without a unified federal framework, the current patchwork of conflicting state and federal laws forces providers to navigate a regulatory maze, driving up costs and creating legal uncertainty. Oversight should be streamlined into a national, federal framework.

Specifically, we urge ASTP/ONC to consider the difficulties of scaling generative AI (GenAI) in clinical settings. While specific tools like ambient scribes have found an early niche, the broader application of GenAI for record summarization is only now reaching the core Electronic Health Records (EHR) layer. The absence of clear regulatory guardrails, illustrated by high-profile litigation such as the recent class-action suits regarding third-party data tracking and consent, creates a climate of institutional risk aversion. Without federal guidance that harmonizes patient autonomy with the technical realities of LLM data flows, health systems like Ascension may feel they must delay scaling these technologies to avoid the liability of implied versus express consent.

Furthermore, we urge regulators to address the dual barriers of emerging cyber risk and data liquidity. The introduction of generative models into clinical workflows creates novel attack vectors and downstream vulnerabilities that are not yet fully understood or mitigated by legacy security frameworks. These risks, coupled with the slow pace of AI validation driven by the risk mismatch between privacy-preserving rules and the need for high-quality EHR and imaging datasets, significantly extend development lifecycles. To prevent a chilling effect on innovation, federal action must prioritize the creation of safe-harbor pathways for data validation and standardized cyber-defense protocols tailored for generative architectures.

Crucially, this framework must include formal recognition by HHS in determining reimbursement for the substantial costs associated with onboarding, vetting, and integration, as well as the ongoing IT support and maintenance required to sustain these complex systems. By aligning financial incentives with the technical realities of implementation, the federal government can ensure that the burden of pioneering safe AI does not fall solely on the providers.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

Ascension urges HHS to prioritize the explicit integration of AI-driven clinical summarization into federal interoperability mandates and the ONC Health IT Certification Program. While current technical strategies focus on the transport of data, they ignore the cognitive overload at the point of care. Even in a future where data is perfectly normalized, the resulting longitudinal record is often too vast for a

clinician to manually synthesize. AI that consumes the record is the essential catalyst for achieving the ultimate goal of interoperability: ensuring clinicians actually know their patients.

HHS should specifically revisit 45 CFR § 170.315 to pivot certification focus away from rigid, prescriptive data-capture formats (like CCDA XML or specific JSON structures) toward the accuracy and utility of AI-generated clinical outputs. If generative AI can accurately summarize patient history from less-normalized data, strict adherence to legacy terminology standards becomes a lower priority. By shifting the regulatory focus under 45 CFR Part 170 toward output fidelity rather than input structure, HHS can reduce the development time currently wasted on normalization that may no longer be necessary for high-functioning AI substrates.

Additionally, in order to incentivize adoption, HHS must also address the consent-driven throttle on scaling. Uncertain consensus approaches to patient education and AI consent create significant legal risks that stifle institutional adoption. HHS should use its convening power to establish "Safe Harbor" consent frameworks.

HHS should also modernize the Information Blocking framework under 45 CFR Part 171 by explicitly clarifying that the exchange of AI-validated performance metrics and quality assessments for clinical improvement does not constitute a regulatory violation. Current ambiguity surrounding the definition of interference in 45 CFR § 171.103 creates significant administrative hesitation, preventing developers and providers from sharing the transparency data essential for safe implementation. By expanding the Health IT Performance Exception (§ 171.205) to formally include the proactive sharing of drift analysis and validation results, HHS can eliminate the perceived legal risks that currently stall data flow, ensuring these exchanges are recognized as protected Health Care Operations that build necessary clinical confidence.

In addition, federal reimbursement frameworks must account for the full lifecycle of AI integration. This includes not only the initial capital for onboarding and clinical vetting but also the continuous operational costs of maintenance and IT support. Aligning financial incentives with these technical realities ensures that the financial burden of pioneering safe, scalable AI is shared by the federal government, rather than borne exclusively by providers. Furthermore, as GenAI for record summarization begins to scale within core EHR platforms, payment policy under the CMS Medicare Physician Fee Schedule (42 CFR Part 410) should be augmented to recognize AI-enabled synthesis as a component of complex medical decision-making.

A final consideration for HHS is the implementation of a mandatory review period for AI-generated or AI-augmented health data to ensure clinical accuracy and patient safety. The buffer would allow providers to validate machine-generated insights, preventing the dissemination of unverified information that lacks necessary clinical context or professional assessment.

Without a clear signal of support from ASTP on standards for privacy and clinical accuracy, industry initiatives will remain fragmented and slow to connect the synergy between data exchange and AI-driven insights.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

As noted in Ascension's response above, primary concerns involve the authenticity of patient consent and the responsibility of Intent-Driven Network (IDNs) to document that consent with high granularity.

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

Ascension recommends that HHS take an active role in establishing a clinical AI governance framework by leveraging Conditions of Participation and quality programs to codify expectations for human-in-the-loop oversight and local institutional governance. To serve as a national benchmark, the agency should align its internal AI adoption with these same principles, effectively translating high-level theory into concrete program requirements and guidance.

For AI applications that fall outside traditional medical device classifications, it is essential that HHS standardize evaluation methodologies across the entire lifecycle, ranging from pre-deployment robustness testing and bias audits to rigorous post-deployment monitoring for performance drift and calibration. These evaluation frameworks must remain human-centered, specifically measuring the technology's impact on clinician cognitive load and the integrity of existing clinical workflows.

Furthermore, HHS should lead the development of standardized disclosure and communication models to ensure authentic patient consent, with the goal of incrementally integrating these standards into official patient rights and informed consent guidance. To support the implementation of these processes, HHS should provide developers and health systems with practical tools via high-impact mechanisms, including grants and cooperative agreements for implementation science, prize competitions for the development of open-source monitoring utilities, and targeted contractual requirements. This coordinated approach would provide the healthcare sector with a stable, actionable roadmap that maintains safety and trust as the foundational pillars of clinical AI integration.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

Ascension urges HHS to define the regulatory floor for safety, transparency, and clinical outcomes. By establishing these essential baseline standards, it can provide a predictable framework that ensures all AI tools meet a minimum threshold of efficacy without stifling progress. It is also critical that HHS aligns these federal benchmarks with private sector accreditation bodies. Rather than creating disparate or redundant layers of oversight, a harmonized approach allows industry-driven certification and credentialing to serve as a robust extension of federal safety goals. This alignment reduces administrative friction for healthcare innovators and ensures that the private sector can lead in technical testing while remaining grounded in the safety and transparency requirements mandated by the agency.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

In assessing the national landscape of AI deployment, it is evident to Ascension that these tools have achieved significant success in areas where they can streamline high-volume tasks and optimize resource allocation. Specifically, AI has met or exceeded performance and cost expectations in imaging and diagnostics, as well as in predictive analytics focused on hospital readmissions, length of stay, and case prioritization. These applications have allowed health systems to focus critical resources on high-risk patients and improve case review rates, effectively increasing clinical throughput and efficiency without the need for additional staffing.

However, the transition of AI from controlled environments to broad clinical practice has revealed notable shortcomings. Performance often degrades when tools are introduced to new patient populations, differing EHR systems, or varied local coding practices. Furthermore, many current AI models fail to adequately account for equity and bias risks, potentially exacerbating disparities in care. From a workflow perspective, the high frequency of generated alerts has contributed to clinician fatigue, while a parallel concern exists regarding automation bias, where over-reliance on AI recommendations can lead to diagnostic errors or near-misses.

Looking forward, the greatest potential to improve outcomes and reduce costs lies in developing novel tools that move beyond narrow task-specific functions toward more adaptive and equitable systems. High-potential innovations include AI that can maintain performance across diverse data environments and tools that provide deeper insights into quality by contextualizing clinical data with social determinants of health. By focusing on these areas, the healthcare sector can transition toward a more integrated model of AI that supports clinician decision-making while actively mitigating the risks of bias and technical instability.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

At Ascension, the adoption of clinical AI is driven by our Chief Medical Information Officer, who works in lockstep with our clinical executive leadership, including the Chief Clinical and Nursing Officers. This unified front is necessary because we view AI as a fundamental shift in care delivery rather than a simple IT upgrade. Our focus is on building the rigorous governance, funding models, and oversight frameworks required to ensure these tools are deployed responsibly and sustainably. In addition, we have a multi-disciplinary AI Governance Committee composed of leaders from across Ascension including Technology, Clinical Operations, Operations, Finance and Data Science, Mission, Legal, Compliance, Risk, Human Resources, Marketing and Digital Experience, and Clinical Transformation.

While we are eager to innovate, we face industry-wide hurdles regarding the complexity of health data. The inherent variability in clinical coding and the nuances of real-world patient data require significant effort to refine into the high-quality, reliable inputs that AI tools demand for peak performance. Additionally, we often navigate a fragmented vendor landscape where limited interoperability creates silos, making it a challenge to integrate new tools seamlessly into our existing workflows. Overcoming these barriers requires us to look beyond the initial investment and focus on elevating data standards and formalizing oversight to protect patient safety and enhance the quality of care.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data

standards, and benchmarking tools.

Ascension believes that enhanced interoperability serves as the essential catalyst for expanding market opportunities and accelerating clinical AI development, particularly when focused on core clinical data types such as diagnoses, medications, allergies, vital signs, and clinical notes. By utilizing standardized terminologies—including International Classification of Diseases, 10th Revision (ICD-10) for diagnoses, Systematized Nomenclature of Medicine Clinical Terms (SNOMED CT) for clinical terminology, RxNorm for medications, Logical Observation Identifiers Names and Codes (LOINC) for laboratory results, and Current Procedural Terminology (CPT) for procedures—healthcare can move away from siloed, proprietary data structures toward a unified clinical language. A transition of this nature allows developers to scale their innovations across different health systems more rapidly, as the common language reduces the need for expensive, one-off data mapping exercises.

The adoption of modern data exchange standards, such as Fast Healthcare Interoperability Resources (FHIR), further accelerates this progress by enabling the use of bulk data for model training and external validation. Such an approach allows researchers to move beyond small, localized datasets to create more robust and generalizable AI models that are validated across diverse clinical environments. Furthermore, the integration of high-velocity data from remote monitoring, wearables, and other patient-generated sources provides a significant frontier for innovation. When these diverse data types such as biomarkers, imaging, waveforms, and medical device data are made interoperable, they fuel the development of real-time predictive tools and longitudinal research that can dramatically improve personalized care and overall health outcomes.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Patients and caregivers are increasingly looking to AI to bridge persistent gaps in care coordination, chronic disease management, and general access barriers. There is a strong desire for healthcare to become more proactive and personalized through tools that simplify administrative burdens, such as scheduling appointments, managing referrals, and navigating the complexities of prior authorizations and transitions of care. In the realm of chronic condition management, there is significant interest in AI that can predict flares, recognize early signs of deterioration, or identify potential complications. By providing actionable next steps before a crisis occurs, these technologies have the potential to improve quality of life while reducing emergency department visits, hospital readmissions, and mortality rates. Furthermore, in addition to—and hopefully well before—chronic disease management, AI can be a powerful tool in identifying high risk patient subsets where predictive analytics can prompt early interventions to prevent subsequent health events.

Despite these potential benefits, patients and caregivers harbor significant concerns regarding the speed and nature of AI adoption. Trust remains a primary hurdle, particularly regarding the risk of inaccurate information, clinical errors, and the sensitive nature of health data being collected and shared with external vendors. In addition, a profound worry persists regarding the dehumanization of the patient experience—specifically the fear that sensitive or life-altering information might be delivered via an automated bot rather than a compassionate clinician. These concerns are compounded by broader anxieties regarding patient safety and a perceived lack of control over how AI-driven decisions influence personal health journeys.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care? Are there published findings about the impact of adopted AI tools and their use in clinical care?

Ascension recommends that HHS prioritize research into the development and scalability of trustworthy clinical AI tools that are capable of learning from real-time clinical feedback and overrides. Research must focus on seamless workflow integration, specifically for high-impact areas such as the early detection of patient deterioration, optimization of chronic disease management, and diagnostic support within primary care and emergency settings, to ensure these tools move effectively into practice. Patients desire a healthcare ecosystem that knows them; AI could accelerate the perception and realization of continuity and personalization of care. Moreover, accelerating the adoption of AI requires the establishment of clear federal guidelines on oversight governance to help organizations manage the transition of care and reduce readmission rates.

Current published findings highlight a growing divide between the theoretical potential of AI and its real-world implementation.³ While studies have documented increasingly positive impacts on imaging accuracy, triage speed, and administrative efficiency, there is a significant body of cautionary evidence regarding the operational challenges of these tools. Specifically, research indicates that alert fatigue remains a critical issue, with clinicians overriding AI recommendations at rates as high as 50%.⁴ These workarounds, often driven by inconsistent tool performance or poor integration, can undermine both patient safety and the expected return on investment. Additionally, the financial reality of adoption is complex; upfront integration costs frequently exceed short-term savings,⁵ and a significant workforce unreadiness persists, with data suggesting that up to 85% of clinicians require additional training even after an organization has officially adopted the technology.⁶

Conclusion

We appreciate your consideration of these comments. If you have any questions, or if there is any additional information we can provide, please contact me at mark.howell1@ascension.org.

Sincerely,



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Ascension

³ *Superagency in the Workplace: Empowering People to Unlock AI's Full Potential at Work*, McKinsey & Co. (Oct. 28, 2024), <https://www.mckinsey.com/capabilities/tech-and-ai/our-insights/superagency-in-the-workplace-empowering-people-to-unlock-ais-full-potential-at-work>.

⁴ Ting-Xuan Huang et al., *Strategies to Mitigate Alert Fatigue in Clinical Decision Support Systems: A Scoping Review*, 12 *Healthcare* 1022 (2024), <https://pmc.ncbi.nlm.nih.gov/articles/PMC11105146/>.

⁵ Keziah Joyce et al., *The Impact of Clinical Decision Support Systems Powered by Artificial Intelligence on Healthcare Workforce Outcomes: A Systematic Review*, 8 *NPJ Digit. Med.* 24 (2025), <https://www.nature.com/articles/s41746-025-01722-y>.

⁶ *Global AI in Healthcare Report: Exploring Trends, Challenges, and Future Projections*, Radixweb (Jan. 22, 2024), <https://radixweb.com/global-ai-in-healthcare-report>.